[redacted]

[redacted] [redacted] [redacted]

OT BOOKLET

Name : [redacted] Age : 65Y Sex : M

Address : [redacted] Phone : [redacted]

UHID No. / IP No. : 202602281431/2026022800 93

Department : ORTHOPAEDICS Unit : II

Ward : MOW - Bed No. : 31

DOA : 28/02/2026 Date of OT : 3/2/2026.

Surgeon's Name : [redacted]

[redacted]

ANESTHESIA CONSENT FORM

Patient Name: [redacted] Age & Sex: 65y/m UHID No.: 20260225 431

Department: ORTHOPAEDICS Ward & Bed No.: MO-/31 Diagnosis: Polytrauma & Trauma to chest & abdomen & D/L Knee

Proposed Surgery/Procedure: _____ Anesthesist: _____ Anesthesia Type: _____

I_____, Self/Relative (_____) of_____ have been explained in my mother tongue the risks of the surgery/procedure , advised me of alternative treatments and told me about the expected out come and what could happen if my condition remains untreated . I also understand that anesthesia services are needed for the surgery/procedure.

It has been explained to me that all forms of anesthesia involve some risks and no guarantees or promises can be made concerning the results of my procedure or treatment. Although rare, unexpected severe complications with anesthesia can occur and include the remote possibility of infection, bleeding, drug reactions, drug allergies, blood clots, loss of sensation, loss of limb function, paralysis, respiratory arrest, stroke, brain damage, heart attack or death.

I understand that these risks apply to all forms of anesthesia and that additional or specific risks have been identified which may apply to a specific type of anesthesia. I understand that type(s) of anesthesia service/technique used for my procedure and that is determined by the clinical requirements.

It has been explained to me that sometimes an anesthesia technique which involves the use of local anesthesia, with or without sedation, may not succeed completely and therefore another technique may have to be used including general anesthesia.

I have been explained the risk and complication to my patient individual circumstances.

I do hereby give consent to undergo surgery/procedure under Anesthesia at my own risk. All pros and cons have been explained and discussed with me.

	Name	Signature	Date	Time
Patient/ Patient Surrogate				
Doctor				
Witness				

[redacted]

ନିଶ୍ଚେତକ ସମ୍ମତି ପତ୍ର

ରୋଗୀଙ୍କ ନାମ: [redacted] ବୟସ / ଲିଙ୍ଗ: 65Y/M ଇ. ଏଚ୍. ଆଇଡି ନଂ: 20260223/43

ବିଭାଗ: ORTHOPEDICS ୱାର୍ଡ/ଶଯ୍ୟା: MOW / 31 ଡାଇଗ୍ନୋସିସ୍: Polytrauma to Chest Abdomen b/l Knee

ପ୍ରସ୍ତାବିତ ଅସ୍ତ୍ରୋପଚାର / ପ୍ରଣାଳୀ: _____ ଆନାସେଥିଷ୍: _____ ନିଶ୍ଚେତକ ପ୍ରକାର: _____

ମୁଁ [redacted] ମୋତେ ମାତୃଭାଷାରେ ଅସ୍ତ୍ରୋପଚାର / ପ୍ରଣାଳୀର ବିପଦ ବିଷୟରେ ବର୍ଣ୍ଣନା କରାଯାଇଛି, ମୋତେ ବିକଳ୍ପ ଚିକିତ୍ସା ବିଷୟରେ ପରାମର୍ଶ ଦେଇଛନ୍ତି ଏବଂ ଆଶା କରାଯାଉଥିବାସମ୍ଭାବ୍ୟ ଫଳାଫଳ ଏବଂ ଯଦି ମୋର ଚିକିତ୍ସା ନହୁଏ ତେବେ ଅବସ୍ଥା କ'ଣ ହୋଇପାରେ ବୋଲି ମୋତେ ଜଣାଇ ଦିଆଯାଇଛି | ମୁଁ ମଧ୍ୟ ତାହା ବୁଝିପାରୁଛି ଯେ ଅସ୍ତ୍ରୋପଚାର / ପ୍ରଣାଳୀ ପାଇଁ ନିଶ୍ଚେତକ ସେବା ଆବଶ୍ୟକ |

ଏହା ମୋତେ ବୁଝାଇ ଦିଆଯାଇଛି ଯେ ସମସ୍ତ ପ୍ରକାରର ନିଶ୍ଚେତକରେ କିଛି ବିପଦ ରହିଥାଏ ଏବଂ ଚିକିତ୍ସା ପଦ୍ଧତି କିମ୍ବା ଚିକିତ୍ସା ର ଫଳାଫଳ ବିଷୟରେ କୌଣସି ଗ୍ୟାରେଣ୍ଟି କିମ୍ବା ପ୍ରତିଶ୍ରୁତି କରାଯାଇପାରିବ ନାହିଁ | ଯଦିଓ ବିରଳ, ନିଶ୍ଚେତକ ସହିତ ଅପ୍ରତ୍ୟାଶିତ ଗୁରୁତର ଜଟିଳତା ହୋଇପାରେ ଏବଂ ସଂକ୍ରମଣ, ରକ୍ତସ୍ରାବ, ଡ୍ରଗ୍ ପ୍ରତିକ୍ରିୟା, ଡ୍ରଗ୍ ଆଲର୍ଜି, ରକ୍ତ ଜମାଟ ବାନ୍ଧିବା, ସମ୍ବେଦନଶୀଳତା ହଜାଇବା, ଅଙ୍ଗପ୍ରତ୍ୟଙ୍ଗ କାର୍ଯ୍ୟ ହରାଇବା, ପକ୍ଷାଘାତ, ଶ୍ୱାସକ୍ରିୟା ବନ୍ଦ, ଷ୍ଟ୍ରୋକ, ମସ୍ତିଷ୍କ କ୍ଷତି, ହୃଦଘାତ କିମ୍ବା ମୃତ୍ୟୁ ମୁଁ ବୁଝିପାରୁଛି ଯେ ଏହି ବିପଦଗୁଡିକ ସମସ୍ତ ପ୍ରକାରର ନିଶ୍ଚେତକ ପାଇଁ ପ୍ରଯୁଜ୍ୟ ଏବଂ ଅତିରିକ୍ତ କିମ୍ବା ନିର୍ଦ୍ଦିଷ୍ଟ ବିପଦଗୁଡିକ ଚିହ୍ନଟ କରାଯାଇଛି ଯାହା ଏକ ନିର୍ଦ୍ଦିଷ୍ଟ ପ୍ରକାରର ନିଶ୍ଚେତକ ପାଇଁ ପ୍ରୟୋଗ ହୋଇପାରେ |

ମୁଁ ବୁଝିପାରୁଛିଯେ ମୋ ପାଇଁ ବ୍ୟବହୃତ ନିଶ୍ଚେତକ ସେବା / କୌଶଳପଦ୍ଧତିର ପ୍ରକାର କ୍ଲିନିକାଲ୍ ଆବଶ୍ୟକତା ଦ୍ୱାରା ନିର୍ଣ୍ଣୟ କରାଯାଏ | ମୋତେ ଏହା ବୁଝାଇ ଦିଆଯାଇଛି ଯେ କେବେକେବେ ଏକ ନିଶ୍ଚେତକକୌଶଳ ଯାହାକି ସ୍ଥାନୀୟ ନିଶ୍ଚେତକର ବ୍ୟବହାର ସହିତ କିମ୍ବା ବିନା ସେତେପନରେ ସମ୍ପୂର୍ଣ୍ଣ ଭାବରେ ସଫଳ ହୋଇପାରିବ ନାହିଁ ଏବଂ ସାଧାରଣ ନିଶ୍ଚେତକ ସହିତ ଅନ୍ୟ ଏକ କୌଶଳ ବ୍ୟବହାର କରିବାକୁ ପଡିପାରେ | ମୋର ରୋଗୀର ବ୍ୟକ୍ତିଗତ ପରିସ୍ଥିତିରେ ମୋତେ ବିପଦ ଏବଂ ଜଟିଳତା ବର୍ଣ୍ଣନା କରାଯାଇଛି |

ମୁଁ ନିଜ ବିପଦରେ ନିଶ୍ଚେତକ ଅଧୀନରେ ଅସ୍ତ୍ରୋପଚାର / ପ୍ରଣାଳୀ କରିବାକୁ ସମ୍ମତି ଦେଉଛି | ସମସ୍ତ ସମ୍ଭାବ୍ୟ ଫଳାଫଳ ମୋ ସହିତ ଆଲୋଚନା କରାଯାଇଛି |

	ନାମ	ଦସ୍ତଖତ	ତାରିଖ	ସମୟ
ରୋଗୀ / ପରିବାର	[redacted]	[redacted]	2/3/2026	9:30 PM
ସାକ୍ଷୀ		[redacted]		
ଡାକ୍ତର	[redacted]	[redacted]	2/3/2026	4.30 PM

[redacted]OT/FF/OTB-01

[redacted]

ସର୍ଜରୀ / ପ୍ରକ୍ରିୟା ପାଇଁ ନିମିତ୍ତ ସମ୍ମତି ପତ୍ର

ରୋଗୀଙ୍କ ନାମ: [redacted] ବୟସ / ଲିଙ୍ଗ: 65Y/M ଉ. ଏଚ୍. ଆଇଡି ନଂ: 20260228/123

ବିଭାଗ: ORTHOPAEDICS ୱାର୍ଡ/ଶଯ୍ୟା: MOW/ 31 ଡାଇଗ୍ନୋସିସ୍: POLYTRAUMA to Check abdomen injury

ଶଲ୍ୟଚିକିତ୍ସା ପଦ୍ଧତି ଶଲ୍ୟ କ୍ରିୟା: _____ ସର୍ଜନ ଙ୍କ ନାମ: _____

ମୁଁ [redacted] ସ୍ୱୟଂ/ସମ୍ପର୍କୀୟ (_____)

_____ ର, ମୋତେ ମୋ ମାତୃଭାଷାରେ ବ୍ୟାଖ୍ୟା କରାଯାଇଛି ଯେ,

- ମୋର/ ସମ୍ପର୍କୀୟର ଚିକିତ୍ସା ଅବସ୍ଥା, ପ୍ରସ୍ତାବିତ ଚିକିତ୍ସା / ପ୍ରଣାଳୀ ଏବଂ ଯୋଜନାବଦ୍ଧ ଚିକିତ୍ସା / ପ୍ରଣାଳୀ ସହିତ ଜଡିତ ବିପଦଗୁଡିକ ମୋ ପାଇଁ ନିର୍ଦ୍ଦିଷ୍ଟ ବିପଦ ଏବଂ ସେମାନଙ୍କର ସମ୍ଭାବ୍ୟ ଫଳାଫଳକୁ ଡାକ୍ତର ସ୍ପଷ୍ଟ କରିଛନ୍ତି ।
- ଡାକ୍ତର ଅନ୍ୟାନ୍ୟ ପ୍ରାସଙ୍ଗିକ / ବିକଳ୍ପ ଚିକିତ୍ସା ଏବଂ ସେମାନଙ୍କ ସହ ଜଡିତ ବିପଦ ବିଷୟରେ ବର୍ଣ୍ଣନା କରିଛନ୍ତି। ପଦ୍ଧତି ନ ହେବାର ଆଶଙ୍କା ମଧ୍ୟ ଡାକ୍ତର ସ୍ପଷ୍ଟ କରିଛନ୍ତି। ମୋତେ ଦ୍ୱିତୀୟ ମତାମତ ନେବାକୁ ପସନ୍ଦ ଦିଆଯାଇଛି।
- ମୁଁ ବୁଝିପାରୁଛି ଯେ ଚିକିତ୍ସା / ପ୍ରଣାଳୀରେ ରକ୍ତ / ରକ୍ତ ଉତ୍ପାଦ ଦେହାନ୍ତରଣ ଅନ୍ତର୍ଭୁକ୍ତ ହୋଇପାରେ ।
- ମୁଁ ବୁଝିପାରୁଛି ଯେ ଅସ୍ତ୍ରୋପଚାର ସମୟରେ ଯଦି ଅଙ୍ଗ କିମ୍ବା ଟିସୁ ଅପସାରଣ କରାଯାଏ ଯାହା ପରୀକ୍ଷା ପାଇଁ ରଖାଯାଇପାରିବ ଏବଂ ନିୟାମକ ବ୍ୟବସ୍ଥା ଅନୁଯାୟୀ ଡାକ୍ତରଖାନା ଦ୍ୱାରା ସମ୍ବେଦନଶୀଳ ଭାବରେ ନିଷ୍କାସନ କରାଯିବ।
- ଡାକ୍ତର ବ୍ୟାଖ୍ୟା କରିଛନ୍ତି ଏବଂ ମୋତେ ସହମତ କରାଯାଇଛି ଯେ ଚିକିତ୍ସା / ପ୍ରକ୍ରିୟା ସମୟରେ ଯଦି ତୁରନ୍ତ ଜୀବନ ପ୍ରତି ବିପଦ ଘଟେ, ତେବେ ସେଗୁଡିକ ପ୍ରଚଳିତ ଚିକିତ୍ସା ନିୟମ ଅନୁଯାୟୀ କରାଯିବ।
- ଏହା ମୋତେ ବୁଝାଇ ଦିଆଯାଇଛି ଯେ, ଅପରେସନ୍ / ପ୍ରଣାଳୀରେ / କ୍ରିୟା ପରବର୍ତ୍ତୀ ସମୟରେ, ଅପ୍ରତ୍ୟାଶିତ ଅବସ୍ଥା ପ୍ରକାଶ ପାଇପାରେ କିମ୍ବା ସାମ୍ନାକୁ ଆସିପାରେ ଯାହା ଜରୁରୀ ସର୍ଜିକାଲ୍ କ୍ରିୟା ଅନ୍ୟାନ୍ୟ ପ୍ରକ୍ରିୟା ଆବଶ୍ୟକ କରେ ଯାହା ବ୍ୟତୀତ କ୍ରିୟା ଭିନ୍ନ ଅଟେ । ଏହି ପରିପ୍ରେକ୍ଷୀ ରେ, ମୁଁ ଅତିରିକ୍ତ ସର୍ଜିକାଲ୍ କ୍ରିୟା ଅନ୍ୟାନ୍ୟ ପ୍ରକ୍ରିୟା କରିବାକୁ ଅନୁମତି ଦେଉଛି ଯେହେତୁ ସେ ମୋ ସ୍ୱାର୍ଥରେ ଆବଶ୍ୟକ ମନେ କରନ୍ତି ।
- ଡାକ୍ତରୀ, ବୈଜ୍ଞାନିକ କ୍ରିୟା ଶିକ୍ଷାଗତ ତଥା ରିସର୍ଚ୍ଚ ଉଦ୍ଦେଶ୍ୟରେ ମୋ ଶରୀରର ଉପଯୁକ୍ତ ଅଂଶ ଅନ୍ତର୍ଭୁକ୍ତ କରି ଅପରେସନ୍ (ଗୁଡିକ) କ୍ରିୟା ପ୍ରଣାଳୀ (ଗୁଡିକ) ର କୌଣସି ଫଟୋଗ୍ରାଫି କ୍ରିୟା ଟେଲିଭିଜନ୍ କରିବାକୁ ସହମତ । ତଥାପି ଡାକ୍ତରଖାନା ଦ୍ୱାରା ଉପଯୁକ୍ତ ସତର୍କତା ଅବଲମ୍ବନ କରାଯିବ ଯାହା ମୋର ପରିଚୟ କୌଣସି ସ୍ଥାନରେ ପ୍ରକାଶ ପାଇବ ନାହିଁ ।

ମୁଁ ଏଠାରେ ଅସ୍ତ୍ରୋପଚାର / ପ୍ରଣାଳୀ କରିବାକୁ ସମ୍ମତି ଦେଉଛି ଯାହା ମୋ ଦ୍ୱାରା ସମ୍ପୂର୍ଣ୍ଣ ଭାବରେ ବ୍ୟାଖ୍ୟା କରାଯାଇଛି ଏବଂ ଏହା ସ୍ୱେଚ୍ଛାକୃତ ଏବଂ ସଂରକ୍ଷଣ ବିନା ସ୍ଥିର ମନ ସହିତ ସମ୍ମତି ପ୍ରଦାନ କରୁଛି ।

	ନାମ	ଦସ୍ତଖତ	ତାରିଖ	ସମୟ
ରୋଗୀ / ସମ୍ପର୍କୀୟ	[redacted]	[redacted]	2/3/2026	9:30 PM
ଡାକ୍ତର	[redacted]		3/3/26	[illegible]
ସାକ୍ଷୀ	[redacted]		2/3/2026	9:30 PM

[redacted]

PRE-OPERATIVE HANDING-TAKING OVER FORM

ACTIVITIES			REMARKS
CHECK (IN WARD)			
1. CONSENT FORM FILLED	YES	NO	
2. ANESTHETIC PREPARATION	YES	NO	
• Fasting	YES	NO	
• Dentures	YES	NO	
• Nail polish	YES	NO	
• Ornaments	YES	NO	
3. SITE TO BE PREPARED			
• Shave(Skin Preparation)	YES	NO	
• Antiseptic dressing	YES	NO	
• No hair clip of metal pins	YES	NO	
• Two plats for long hair	YES	NO	
• Urinary catheter in SITU	YES	NO	
4. SIDE OF OPERATION	Rt.	Lt.	NA
5. INVESTIGATION REPORTS			
• PATHOLOGY	YES	NO	ECG-
• X-RAY /SONOGRAMS	YES	NO	Chest x-ray-
• Scans	YES	NO	CT/MRI
• ECG/Echo cardiogram	YES	NO	
• Blood transfusion	YES	NO	Pantnmrs —
• Blood reserved with particulars	YES	NO	
6.DRUGS ADMINISTERED IF ANY	YES	NO	INJ PAN 40(1vial-IV).
7.ALLERGIC TO MEDICINES	YES	NO	INJ TT. 1 Ample - IM.
8.FOR CATHLAB CASES:	YES	NO	INJ. xylocaine 0.5% - skin test.
• IV LINE	Yes		
• Empty bladder before sending	Yes		
• Special instruction			INJ - Biocetozo-T2 (125 mg IV)
CHECK 2 IN (PRO-OPERATIVE ROOM)	YES	NO	
9. PATIENT IDENTITY CROSS CHECKED	YES	NO	
10. INVESTIGATION- COMPLETE/INCOMPLETE	YES	NO	
11. CROSS CHECKED SITE/SIDE TO BE OPERATED	YES	NO	
12. HANDED OVER BY	SIGNATURE NAME:		[redacted]
13. TAKEN OVER BY	SIGNATURE NAME:		
14. COUNTER SIGN BY ANAESTHETIST/ SURGEON	SIGNATURE NAME:		

Before induction of anaesthesia

(with at least nurse and anaesthetist)

Has the patient confirmed his/her identity, site, procedure, and consent?

- ☐ Yes

Is the site marked?

- ☐ Yes
- ☐ Not applicable

Is the anaesthesia machine and medication check complete?

- ☐ Yes

Is the pulse oximeter on the patient and functioning?

- ☐ Yes

Does the patient have a:

Known allergy?

- ☐ No
- ☐ Yes

Difficult airway or aspiration risk?

- ☐ No
- ☐ Yes, and equipment/assistance available

Risk of >500ml blood loss (7ml/kg in children) ?

- ☐ No
- ☐ Yes, and two IVs/central access and fluids planned

Before skin incision

(with nurse, anaesthetist and surgeon)

- ☐ Confirm all team members have introduced themselves by name and role.
- ☐ Confirm the patient's name, procedure, and where the incision will be made.

Has antibiotic prophylaxis been given within the last 60 minutes?

- ☐ Yes
- ☐ Not applicable

Anticipated Critical Events

To Surgeon:

- ☐ What are the critical or non-routine steps?
- ☐ How long will the case take?
- ☐ What is the anticipated blood loss?

To Anaesthetist:

- ☐ Are there any patient-specific concerns?

To Nursing Team:

- ☐ Has sterility (including indicator results) been confirmed?
- ☐ Are there equipment issues or any concerns?

Is essential imaging displayed?

- ☐ Yes
- ☐ Not applicable

Before patient leaves operating room

(with nurse, anaesthetist and surgeon)

Nurse Verbally Confirms:

- ☐ The name of the procedure
- ☐ Completion of instrument, sponge and needle counts
- ☐ Specimen labelling (read specimen labels aloud, including patient name)
- ☐ Whether there are any equipment problems to be addressed

To Surgeon, Anaesthetist and Nurse:

- ☐ What are the key concerns for recovery and management of this patient ?

Signature & Seal :

1. OT Nurse :

2. Surgeon :

3. Anae

[redacted]

OT CHARGE SHEET

Patient's Category : [redacted]	Diagnosis : OA knee CN	Dept. :
		Bed No. :
2.	Surgery : TKR CRA Knee	Microscopic Time :
Anaesthetist Name [redacted]		Start Time :
Asst. Surgeon Name		End Time :
Sister's Name [redacted]		C-Arm Time :

Anaesthesia Consumables	Qty	Name	Qty	Surgical Consumables	Qty
I.V.Cannula	—	Inj. Ketamine		Cap & Mask	—
IV Set	—	Inj. Propofol		Gloves	—
B.T.Set/Micro Set		Inj. Midazolam		Foleys2 ways	—
3Ways		Halothane		Urosac Bag	—
3Ways+10cm Extension		Sevoflurane		Urometer	
3Ways+100cm Extension		Isoflurane		Xylocaine Jelly	
P.M.Line		Inj. Scoline		Vaccum Drain	
Paediatric Drip Set		Inj. Pancuronium		Surgical Mesh	
ECG Electrodes		Inj. Vecuronium		Absorbant Gel	
Ventilator Circuit		Inj. Rocuronium		Bone Wax	
E.T.Tube		Inj. Atracurium		Umbilical Cottontape	
H.M.E+Filter		Inj. Atropine		Chestdrainage Tube	
H.M.E600		Inj. Glycopyrrolate		Corrugated Drain	
Suction Catheters		Inj.Neostegmine-1ml		Suction Set	
Ryle's Tube		Inj. Metoclopramide		Suture	—
I.P.Needles		Inj. Ondansetron		1	
Epidura IPack		Inj. Ranitidine		2	
Infant Feeding Tube		Inj. Pantoprazole		3	
Syringes		Inj. Hydrocortisone		4	
1ml		Inj. MethylPrednisolone		5	
2ml		Inj. Dexamethasone	—	Antibiotics	—
5ml	2	Inj. Derphyline		1	
10ml	2	Inj. Terbutaline		2	
20ml		Inj. Lasix		3	
50ml		Inj. Xylocaine2%30ml		4	
I.V.Fluids		Inj. Bupivacaine0.25%		Implants	—
1 NS	(2+1)	Inj. Bupivacaine0.5%			
2 RL	(1)	Inj. BupivacaineHeavy0.5%	—		
3		Inj. Lignocaine	—		
Mannitol					
Analgesics					
Inj. Morphine					
Inj. Fentanyl					
Inj. Pethidine					
Inj. Diclofenac					
Inj. Tramadol					
Inj. Eptoin					
Inj. Mephentine					
Inj. Thiopentone					

[redacted] [redacted] [redacted]

Pharmacist's Signature

[redacted]

ANAESTHESIA RECORD

PROCEDURE NAME : TKR (R) knee LSA

PRE-OPERATIVE FINDING : BP: 118/76 mmHg PR: 82/min SPO₂: 100% RR: 16/min

ANAESTHETIC TECHNIQUE:

1. GENERAL

A. Pre-medication

B. Induction

C. Intubation: Oral/ Nasal

Laryngoscope view CORMACK LEHANE

D. Endotracheal Tube size Fixed at:

E. Maintenance IPPV Ratio of N_2O & O_2

F. Maintenance: Inhalation Agents

G. Ventilator settings – VC/PC TV- PR-RR- PEEP- PMAX-

H. Relaxant used :

I. Reversal :

2. REGIONAL BLOCK : Spinal/epidural/regional

LA at a 25G Quincke's needle was introduced into L3-L4 space in sitting position. 2ml Rexem 0.1ml + 2ml Bupivacaine heavy 3ml was given IT after seeing CSF free flow

Notes (if any) :

[redacted]

Procedure: TKR Left knee

Induction time: 9.40 a.m

Incision time: 9.55 a.m

Completion time: 12.00

Monitors

NIBP	☑
ECG	☑
PULSE	☑
IBP	☐
TEMP	☐
ETCO2	☐
CATHETER	☐
SpO_2	☑

Position:

Supine ☑ Lateral ☐ Prone ☐ Lithotomy ☐

Trendelenburg ☐ Rev. Trendelenburg ☐ Others ☐

MABL: 650ml

Intra op fluid: 1050ml

Input:

NS	RL	Blood	Others
1+1	1		

Output:

Intra OP drugs:

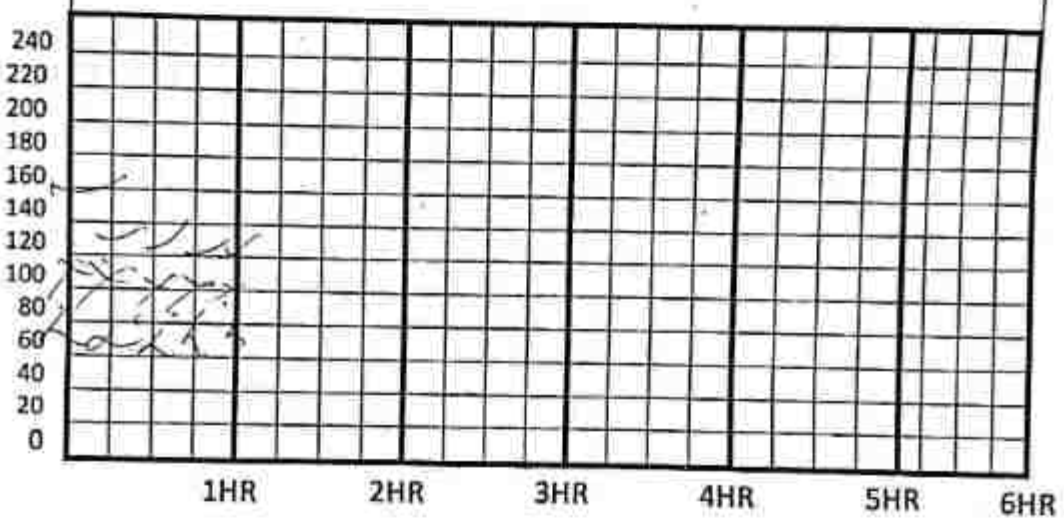
Blood loss:

Intra-OP Procedure:

[redacted]

AGENTS

USED	1 2% Xylocaine heavy + 2% Dexa
RELAXANT	2
INHALATION	3
ANAESTHETICS	4



Recovery Notes:

Consciousness – Awake/Drowsy/Sleeping

Respiration: Spontaneous/IPPV Regular/Irregular

PR: 84 /Min BP: 118/74 mmHg

Time of Arrival:

Reflexes: Good/Satisfactory/Poor

TV Adequate/Inadequate

RR: 16 /Min SpO_2: 100

Post Op Order

Position: Recovery/Propped Up/Supine

I/O Charting: [illegible] Vitals Charting: [illegible] Oxygen: [illegible]

Analgesic: [illegible]

IV Fluids [illegible]

[redacted]

[redacted] of Anaesthetist

[redacted]/OT/FF/OTB-01

[redacted]

O.T. NOTE

Procedure / Surgery : TKR (R) knee Type of Anaesthesia : LSA

OT Start Time : 9.40 a.m OT End Time : 12.00 pm Total Duration : 2hr 20m

Surgeon : 1. Dr. N.K Parigrahi Anaesthesiologist : 1. Dr. B. Mohapatra

2. 2.

Scrub Nurse : Anita OT Nurse : Rajesh

Operative Diagnosis :	OA (R) knee		
Post-Operative Diagnosis :	s/p TKR		
Operative Procedure Proposed :	"	Typeof Procedure	☑ Major ☐ Minor
Operative Procedure Executed :	"		
SpecimenForwardedto Pathology Departmentfor Examination :	—		
Skin Preparation :	Done		
FINDINGS : Record Condition of all organ Examined :	—		
PROCEDURE : Include : Incision : Ligatures :	Done.		

[redacted]

[redacted]

Sutures	Done.
Specimen removed	—
Drainage	—
Sponge Count	—
Closure	Done.
Blood Loss	Minimal
Operating Time	2hr.
Post OP - Surgical Orders	Solay (B) kum AP Cat
X - Ray/Histopathology/C-S/Other	"

	Name	Signature	Date	Time
Surgeon/ Asst.Surgeon	[redacted]	[redacted]	3/3/26	01:21:2
Anaesthetist/ Asst.Anaesthetist	[redacted]	[redacted]	3/3/26	[illegible]

[redacted]

OT NOTE: On 3/3/26 U all asepti. condn U SA Pt - lied supine scrub paint drap done cut with medial parapatellar approach skin incision given joint opened and femur fibrocut taken & replaced and c/o OA(R) knee TKR

POST- OPERATIVE NOTE/ADVICE :

NBO 6hrs
Inf (1) gm
(1) NS.

Play (R) knee splint

I/O chart

Inj BLOCKRAZO 1.2 IV bolus
Inj AMIKACIN IV bolus
Inj METROGYL 500 IV TDS
Inj DYNAPAR AQ IV bolus
Inj PAN 40 IV OD

T. BIJOHEA

[redacted]

Signat

Date :

Time :

Time :

[redacted]OT/FF/OTB-01

[redacted]

POST-OPERATIVE CARE CHECKLIST

SL. NO.	DESCRIPTION	YES	NO	REMARKS
1	Patient identified correctly.	✓		
2	Patient transferred safely from trolley to the post-op. bed			
3	Patient covered with sheet/blanket	✓		
4	Comfortable position given			
5	Oxygen/ Monitor connected (if ordered)			
6	Drainage for any bleeding or blockage checked.	✓		
7	I.V. line checked (functioning as per the doctor's order)			
8	Vital signs checked & recorded		✓	
9	Drainage tube connected properly			
10	Observed wound for any bleeding			
11	Administered antibiotics or any other drugs as per order	✓		
12	Intake and output chart maintained			
13	Continue recording vital signs as advised	✓		
14	Specific instruction (if any)	N/A		

Name	Signature	Date	Time
OT Nurse - [redacted]	[redacted]	3/3/26	[illegible]
Ward Nurse - [redacted]	[redacted]	3/3/26	1:05pm

[redacted]/OT/FF/OTB-01

[redacted]

POST-OP MONITORING FORM

PATIENT NAME : [redacted] AGE/SEX : 65/M DATE/TIME : 3/3/26

UHID/IPD NO. : 2026022800 93 WARD/BED NO : Ward 31

NAME OF SURGERY : of KR crack knee TYPE OF ANAESTHESIA : LSA

PATIENT CONDITION ON ARRIVAL

CONDITION : Conscious / Semiconscious / Unconscious

POSITION : Supine EXTUBATED: Yes / No

OXYGEN : Ventilator / T-Piece / Mask / Cannula

GENERALCONDITION : Good / Satisfactory / Poor

VITALS :- TEMP : Afebrile RR : 16/min SPO2: 100%

HR : 86/min BP : 118/76 mmHg PAINSCORE: 0/10

POST-OP INSTRUCTIONS :

NBM : Yes POSITION: Supine

OXYGEN/ VENTILATION : Yes IV FLUIDS: NS/RL @ 125

BLOODTRANSFUSION : ANALGESICS: Tab

OTHERS :

MONITORING BY NURSING STAFF - POST OPERATIVE RECOVERY ROOM :

TIME	CONSCIOUSNESS	TEMP.	RR	SPO2	HR	BP	PAIN SCORE	NURSE'S SIGN
12:45	✓	98.6°F	18/m	100%	80/min	120/90 mmHg	0	[redacted]

Inform SOS If,

- ☐ Altered Sensorium
- ☐ Breathing difficulty
- ☐ Falling BP
- ☐ Bleeding
- ☐ Intractable vomiting
- ☐ Falling urine output
- ☐ Intractable pain
- ☐ Seizure
- ☐ Other

Post Operative events/ complications and management (If any) :

[redacted]

SIGN OF NURSING I/C [redacted] ANESTHESIST

[redacted]

SEDATION RECOVERY SHEET

Patient Sign		SS	Score
Consciousness	• Awake, responds easily, A & Ox3 or returned to baseline • Responds readily, but easily falls asleep • Arousable, but not readily • Not responding	3 2 1 0	3
Respiratory	• Breathes easily with adequate volume • Slightly decreased rate and/or volume • Labored or limited respiration • Apnea or inadequate ventilation	3 2 1 0	3
Circulatory	• BP and pulse within baseline limits • BP and pulse approaching baseline limits • Abnormally high or low BP and/or abnormally fast or slow pulse	2 1 0	2
Voluntary movement	• Able to move extremities voluntarily or on command or returned to baseline • Voluntary movement - no purposeful • Unable to lift head or move extremities	2 1 0	2
	Total Score	10	10

Shift To : ICU / HDU / WARD

SIGI [redacted]

PATIENT TRANSFER DETAILS

Shifted To: ICU / HDU / WARD

DATE / TIME : 3/3/26

STAFF NURSE	FULL NAME	SIGNATURE	DATE & TIME
Handed Over By	[redacted]	[redacted]	3/3/26 at 12:45
Received By	[redacted]	[redacted]	3/3/26

[redacted]/OT/FF/OTB-01

[redacted]

CHECK BEFORE PATIENT TRANSFER

TICK THE FOLLOWING

1. CONSENT FILLED UP :	YES	✓	NO	
2. OT NOTE :	YES	✓	NO	
3. POST OP MONITORING :	YES	✓	NO	
4. TRANSFER NOTE :	YES	✓	NO	
5. PROPER SIGN. & SEAL :	YES	✓	NO	

Checked by : [redacted]